

For the use of a Registered Medical Practitioner or Hospital or a Laboratory.

Benzydamine Mouthwash BP 0.15% w/v
Coolora

Active Ingredient: Benzydamine Hydrochloride BP (Each mL contains 1.5 mg of Benzydamine hydrochloride), Sodium Benzoate BP 0.20% w/v as preservative

Excipient: Glycerin BP, Propylene Glycol BP, PEG Hydrogenated Castor Oil (Acrysol K 140), Menthol BP, Flavor Booster, Flavor Dill Relish, Sodium Saccharin BP, Brilliant Blue FCF Color, Tartrazine Yellow Color, Purified Water BP.

Dosage Form: Liquid for use as mouthwash/ gargle.

Product Description: A Light green coloured clear liquid with characteristic taste and fennel odour.

Pharmacodynamics: Pharmacotherapeutic group: Other agents for local oral treatment Benzydamine exerts an antiinflammatory and analgesic action by stabilizing the cellular membrane and inhibiting prostaglandin synthesis.

Mechanism of action: The indazole analogue benzydamine has physicochemical properties and pharmacological activities

which differ from those of the aspirin-like NSAIDs. Unlike aspirin-like NSAIDs which are acids or metabolized to acids, benzydamine is a weak base. In further contrast, benzydamine is a weak inhibitor of the prostaglandin synthesis. Only at concentration of 1mM and above benzydamine effectively inhibits cyclooxygenase and lipooxygenase enzyme activity. It mostly exerts its effects through inhibition of the synthesis of proinflammatory cytokines including tumor necrosis factor-alpha (TNF-) and Interleukin-1 (IL-1) without significantly affecting other proinflammatory (IL-6 and 8) or anti inflammatory cytokines (IL-10, IL-1 receptor antagonist). Further mechanisms of action are hypothesized including the inhibition of the oxidative burst of neutrophils as well s membrane stabilization as demonstrated by the inhibition of granule release from neutrophils and the stabilization of lysosomes. The local anaesthetic activity of the compound has been related to an interaction with cationic channels.

Pharmacodynamic effects: Benzydamine specifically acts on the local mechanisms of inflammation such as pain, oedema or granuloma. Benzydamine topically applied demonstrates anti-inflammatory activity reducing oedema as well as exudate and granuloma formation. Further, it exhibits analgesic properties if pain is caused by an

inflammatory condition and local anaesthetic activity. Hyperthermia, which is indicative of systemic functional involvement, is poorly affected by benzydamine.

Pharmacokinetics:
Absorption: Oral doses of benzydamine are well absorbed and plasma drug concentrations reach a peak fairly rapidly and then decline with a halflife of about 13 hours. Less than 20% of the drug is bound to plasma proteins. Although local drug concentrations are relatively large, the systemic absorption of mouthwash-gargle doses of benzydamine is relatively low compared to oral doses. This low absorption should greatly diminish the potential for any systemic drug side- effects when benzydamine is administered by this route.

Biotransformation: Benzydamine is metabolised primarily by oxidation, conjugation and dealkylation.

Indication: Benzydamine is indicated for the temporary relief of painful conditions of the oral cavity including: tonsillitis, sore throat, radiation mucositis, aphthous ulcers, post-orosurgical and periodontal procedures.

Recommended Dose: Benzydamine solution should generally be used undiluted,

but if stinging occurs it may be diluted with water. The solution should be expelled from the mouth after use. Uninterrupted treatment should not exceed seven days.

Dosage in Adults: When used as a gargle, the usual dose of Benzydamine 0.15% w/v Mouthwash is 15ml (approximately one tablespoon) which should be gargled for at least 30 seconds at 1½ to 3 hourly intervals, as needed. When used as a rinse for oral lesions, the usual dose is again 15ml (approximately one tablespoon) which should be held in the mouth and swirled around for at least 30 seconds, with repeat use every 1½ to 3 hours throughout the day.

Dosage in Children: 5 - 15ml as a gargle if able to do so, or as an oral rinse.

With Impaired Renal Function: Since absorbed benzydamine and its metabolites are excreted in the urine, the possibility of systemic effects should be considered in patients with severe renal impairment.

With Impaired Liver Function Since absorbed benzydamine is highly metabolised in the liver, the possibility of systemic effects should be considered in patients with severe hepatic impairment.

Route of Administration: Oropharyngeal

Contraindication: Benzydamine 0.15% w/v Mouth Wash is contraindicated in

patients with known hypersensitivity to any of the ingredients in the formulation or component of the container.

Warning & Precautions: Benzydamine use is not advisable in patients with hypersensitivity to acetylsalicylic acid or other NSAIDs Caution should be exercised in patients suffering from or with a previous history of bronchial asthma. Benzydamine 0.15% w/v Mouthwash should generally be used undiluted, but if 'stinging' occurs the rinse may be diluted with water. Avoid contact with eyes. Benzydamine 0.15% w/v Mouthwash contains Sodium benzoate which may cause allergic reactions (possibly delayed). It also contains propylene glycol which may cause skin irritation.

Use in Children: Because of the lack of sufficient clinical experience, Benzydamine is not recommended in children under 6 years of age.

Interactions with other medicaments: Not known.

Pregnancy and Lactation:
Pregnancy: Benzydamine 0.15% w/v Mouthwash should not be used in pregnancy Breast-feeding: Benzydamine 0.15% w/v Mouthwash should not be used during lactation unless considered essential by the physician.

Fertility: There is no evidence of a teratogenic effect in animal studies.

Side Effect: The most common side effects are numbness and a stinging feeling in the mouth. Respiratory, thoracic and mediastinal disorders: - Very rare: Laryngospasm or bronchospasm. Gastrointestinal disorders: - Uncommon: Oral numbness (hypoesthesia) and a stinging feeling in the mouth (oral pain). Skin and subcutaneous tissue disorders: - Very rare: pruritus, urticaria, photo-sensitivity reaction and rash, Frequency not known: Angioedema Immune system disorders: - Frequency not known: Anaphylactic reaction which can be potentially lifethreatening. Hypersensitivity reactions.

Symptoms and Treatment of Overdose: Benzydamine 0.15% w/v Mouthwash is unlikely to cause adverse systemic effects, even if accidental ingestion should occur. Intoxication is only expected in case of accidental ingestion of large quantities of benzydamine (> 300mg). Intoxication is only to be expected if large quantities of Benzydamine 0.15% w/v Mouthwash are swallowed. Symptoms associated with ingested overdose of benzydamine are mainly gastrointestinal symptoms and

symptoms of the central nervous system. Most frequent gastrointestinal symptoms are nausea, vomiting, abdominal pain and oesophageal irritation. Symptoms of the central nervous system include dizziness, hallucinations, agitation, anxiety and irritability.

Effect on ability to drive and use machine: Not applicable.

Instruction for Use: Take 15ml and gargle for 20 to 30 second every 1.5 to 3 hours for pain relief. Use undiluted, but if stinging occurs may be diluted with water. For use beyond 7 days should be under medical supervision.

Storage Condition: Protect from direct heat or sunlight. Store below 30°C. Do not freeze.

Shelf Life: 36 months from date of manufacture.
Therapeutic Code: ATC code: A01AD02

Presentation : Transparent PET bottles with white colored cap cum measuring cup with ICPA logo and induction sealing wad.

ICPA

Manufacturer :
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